

إسم المريض رابعياً : غاده خليل حلمي فهم

الحى / المدينة : اميايه

المحافضة : الحيز ه

رقم البطاقة : 28806252101241 رقم التلفون : 01206260969

العمر(تاريخ الميلاد) : 25-06-1988 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور

التعليق على التشخيص : -----

الاسم: د/محمد كيلاي
 الاسم: د/ندي حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جھة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: غاده خليل حلمي فهم National ID: 28806252101241	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

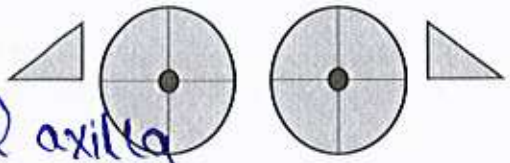
التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM 22/4/2026 G

Name	غادة خليل حليم	MRN	٤٨٦٧
Age	٣٧ سنة	ID	
Menopausal status	Pre menop.	Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC G2	R	L
ER: 8/8	PR: 8/8	HER 2: -ve
Ki67:		
Sonomammography:		
10/2024: MBC to contralateral axilla & bone (by e.Ts)	TNM	
Mets Workup: bone scan. → Free		
Started AI + Zoladex + Zometa 13m		
4/2026: CT: RD of lymph node		

Recommendations: SD of Rest

4/2026: bone scan: met. bone newly developed

For 2nd line.

to Zoladex / T-bone / 26/26

MDT OUTCOME & PLAN

to Zoladex 13m ce. @ Zometa 9.3m

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

23/10/2024, 12:54

code 42 -- Name_A MDT (on Hainnal) -- Name_E MDT (on Hainnal)



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name: **د. محمد علي محمد**

Sex:

3- Medical number:

Female

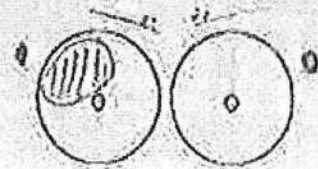
81221055

01/11/2024

23/10/2024

2- ID number:

Patient Age: 36 Y



Family history:

Family history:

Phone:

Tumor:

01002245958

BMI:

HT:

REASON OF NOT

REASON OF NOT

marital status:

Marital status:

NBS Score:

NBS Score:

GROUP:

GROUP:

Date of MDT:

Date of MDT:

Co-morbidity:

Co-morbidity:

Menopausal status:

Menopausal status:

INITIAL STAGE:

INITIAL STAGE:

23/10/2024

Clinical data

Breast Imaging:

Mets Workup:

Mets status: equivocal. It is no lesion

DX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	Her 2 Neu	SISH	10
EXPLANATION	EXPLANATION	EXPLANATION	EXPLANATION	EXPLANATION	EXPLANATION
rt GII & extensive					

T	N	M
4b	3c	1 bone and contralateral axilla

Sonomammography

cesm: rt UOQ 9-12 o'clock 52 x22 mm reaching nipple & 6-7 o'clock 6x3.5 mm

+ve axillary LN +ve supracavicular LN

lt level I,II axillary LN --> mets from rt side

rt iliac bone mixed lytic & sclerotic lesion

PATIENT HISTORY

MDT OUTCOME & PLAN

ONCOLOGY & RX

start 1st line Zoladex + A1, 4 zommetas every 3 months with regular clinical assessment.

SIGNATURE			
MEDICAL ONCOLOGY	<i>[Signature]</i>	RADIOLOGY	<i>[Signature]</i>
SURGERY	<i>[Signature]</i>	PATHOLOGY	<i>[Signature]</i>
Psychiatric	<i>[Signature]</i>	Nutrition	<i>[Signature]</i>

Patient Name : الأستاذة / د. د. خليل حليم فهد

Patient No. : 28806252101241

Path No. : 2300/24

Sex : Female

Age : 36 Y

Referring Doc :

Request No. : 13001432

Request Date : 08/07/2024

Verification Date : 14/07/2024

Financial Category : برنامج التأمين - ممرض

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast 10 O'clock mass.

Nature of Specimen: Tru-cut biopsy, collected at 07/07/2024.

Gross Examination:

Multiple grayish white tissue cores up to 1.5 cm long, totally submitted.

Microscopic Examination:

Histological type: Invasive duct carcinoma.

Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.

Ductal carcinoma in situ: Extensively detected.

Perineural invasion: Not detected.

Angiolymphatic invasion: Not detected.

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with extensive DCIS.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lobna Omar.

Dr. Esraa Magdy.

Verified by :

Dr. Duaa Mohamed

Printing Date : 24/07/2024 13:09:54

Page 1 of 1

Printing By : Marwa Mohamed Younis



Patient's Name: Ghada khalil helmy fahmy
Date: 2026-03-24
Referred by: Prof.Dr. cap+c
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST CT SCAN OF THE
CHEST, ABDOMEN AND PELVIS:**

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma.

Compared to the last study dated 29-11-2025 ,the current study revealed .

(I) Chest:

- Right breast lesion couldn't be appreciated at current study

Regressive course regarding :

- Right enlarged axillary lymph nodes , the largest measures 2 x 1.7cm compared to 2.3 x 2 cm
- Enlarged left axillary lymph nodes , the largest 17 x 10 mm compared to 20 x 19mm.
- Bilateral lower lobar mosaic attenuation of small airway disease.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

Patient's Name: Ghada khalil helmy fahmy
Date: 2026-03-24
Referred by: Prof.Dr. cap+c
Hospital: Goustave Roussey International Egypt

- The liver is enlarged in size showing homogenous CT density no hepatic focal lesions .
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions ,.
- No vertebral structural collapse .

OPINION:

- Regressive course regarding size of bilateral axillary lymphadenopathy .
- Rather stationary course of rest of study

Dr.Reham Khedr, MD

Name : Ghada Khalil Helmy Fahim

Date : 23-03-2026 08:43

Branch : Dokki

Medical No. : 334788

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

ISOTOPIC BONE SCAN REPORT

- **Indication:** 37-year-old female with history of breast carcinoma, presenting with bone aches. Evaluation for possible osseous metastatic disease.
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer (Tc-99m MDP).

FINDINGS:

- Mild abnormal radiotracer uptake is noted along the posterior aspect of the left 10th rib as well as the right sacroiliac region, where metastatic etiology remains a primary consideration.
- No other osseous lesions are identified throughout the remainder of the skeleton.
- Age-related degenerative/arthritis changes are noted in the spine, sternum, both shoulders, and weight-bearing joints.

IMPRESSION:

- Two worrisome osseous lesions as described, likely metastatic, for radiological correlation and clinical follow-up.
- No other osseous lesions elsewhere.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient Name: Ghada Khalil Helmy
Prof. Dr: Center Hormal
Accession: 240905004

Patient ID: 684855
Exam Date: 7/9/2024

^{99m}Tc-MDP whole body bone scan

Desired information:

- ⊙ *Skeletal surveying*

Technique:

- ⊙ *Anterior and posterior Delayed Whole body bone scan performed 2 hours after 1. administration of 20 mCi ^{99m}Tc-MDP;*

Report:

Delayed static phases:

- ⊙ *Active tracer uptake s noted at the left 10th rib posteriorly for CT correlation ?? traumatic ?? otherwise.*
- ⊙ *Mild degenerative changes are noticed in the dorsal & lumbar vertebrae.*
- ⊙ *Otherwise, no hot spot that account for osseous deposits could be detected all over the rest of surveyed skeleton.*
- ⊙ *Both kidneys are visualized.*

CONCLUSION:

- ⊙ *Equivocal left 10th rib lesion for CT correlation to exclude serious nature.*
- ⊙ *Follow up bone scan is advised.*

Best regards;
Dr. Huda Fathy M.D.